LONDON METROPOLITAN UNIVERSITY

Common Neurological Terminologies and Abbreviations

A. <u>Pathologies</u>

- CVA – Cerebrovascular Accident/Stroke
- TBI – Traumatic Brain Injury
- PD – Parkinson's Disease
- PSP – Progressive Supranuclear Palsy
- MSA – Multiple System Atrophy
- TIA – Transient Ischaemic attack
- MS – Multiple Sclerosis
- GBS – Guillain Barre Syndrome
- SCI – Spinal Cord Injury
- CIDP – Chronic Inflammatory Demyelinating Polyneuropathy
- VBI – Vertebro-basilar insufficiency
- BPPV – Benign Paroxysmal Positional Vertigo
- CFS – Chronic Fatigue Syndrome
- ME – Myalgic Encephalopathy/Encephalomyelitis
- VN – Vestibular Neuritis
- TM – Transverse Myelitis
- MI – Myocardial Infarction
- AF – Atrial Fibrillation
- BP – Blood Pressure
- DM – Diabetes Mellitus
- CABG – Coronary Artery Bypass Graft
- PPM – Permanent Pacemaker
- IHD – Ischaemic Heart Disease
- CP – Cerebral Palsy

B. <u>Medical Terminologies</u>

- Infarct – Blood Clot
- Haemorrhage – Bleed
- ICH - Intracranial haemorrhage
- Aneurysm- bursting of an arterial blood vessel
- MRI – Magnetic Resonance Imaging
- CT – Computed Tomography
- EMG – Electromyography
- GCS – Glasgow Coma Scale
- EEG – Electroencephalography
- ECG – Electrocardiography
- CXR – Chest X-ray
- CNS – Central Nervous System
- PNS – Peripheral Nervous System
- WBC – White blood cell
- RBC – Red blood cell
- Hb – Haemoglobin
- HR – Heart Rate
- RR – Respiratory Rate
- SaO_2 – Oxygen Saturation

- DNR – Do not attempt Cardiopulmonary Resuscitation
- CPR – Cardiopulmonary Resuscitation
- UMN – Upper Motor Neuron
- LMN – Lower Motor Neuron
- ReSPECT – Recommended Summary Plan for Emergency Care and Treatment (A summary of personalised recommendations for a person's clinical care in a future emergency when they may not have the capacity to make/express their choices.
- HTN- Hypertension (Increased blood pressure)
- Bradycardia- low heart rate
- Tachycardia- high heart rate

C. <u>Neurological Terminologies</u>

1. Paresis –Paralysis/weakness
2. Hemiparesis – Paralysis/weakness affecting half of the body
3. Tetraparesis – Paralysis/weakness affecting 4 limbs
4. Paraparesis – Weakness affecting the lower limbs
5. Athetosis-slow writhing involuntary movements
6. Proprioception – awareness of the location, movement and action of a body part in space
7. Sensation – the conscious perception of feeling when the body is touched. Can refer to touch, pressure, pain and temperature.
8. Paraesthesia- spontaneous burning, pins and needles sensation not associated with any physical findings
9. Aphasia/dysphasia – an acquired loss or impairment of language - can affect spoken word, understanding, reading and writing.
10. Aphonia-inability to speak or make sound due to damage to the laryngeal nerve
11. Atrophy – wasting of muscles due to lack of use or a neurological disease
12. Hypertrophy – increase size/volume of an organ or tissue. Often used to describe skeletal or cardiac muscle.
13. Athetosis – involuntary movement disorder characterised by slow, purposeless, writhing movements, often more evident in the distal part of a limb.
14. Dysarthria – Slurred speech
15. Muscle Tone – the tension in the relaxed muscle/the resistance felt during passive movement when the muscles are at rest (Ganguly et al., 2021).
16. Hypertonic-increased muscle tone
17. Hypotonic-low muscle tone
18. Flaccid- Lack of muscle tone and strength in a limb
19. Subluxed - a partial dislocation of a joint. Often used to describe the shoulder joint after brain injury/stroke.
20. Tremor – involuntary movement. Can be at rest or during movement.
21. Clonus – regular, repetitive, rhythmic contraction of a muscle subjected to a sudden or maintained stretch (Barrett et al., 2016 cited Ganguly et al., 2021)
22. Spasticity - disordered sensory-motor control, resulting from an UMN lesion, presenting as intermittent or sustained involuntary action (SPASM definition)
23. Contracture- A permanent shortening of a muscle causing a loss of joint range of movement and relaxation of extensor muscles. Abrupt withdrawal of a body part in response to a painful or perceived noxious stimuli

24. Rigidity – Increased stiffness or resistance to movement. Can be lead pipe – constant resistance to movement through range or cogwheel – resistance that stops and starts as the limb is moved through range.
25. Dystonia – A movement disorder that leads to abnormal muscle tone, uncontrolled movements and spasms.
26. Dysmetria- Disturbance of co-ordination due to damage to the cerebellar where a person over or underestimates the movement needed to complete a task
27. Dysphagia – Difficulty with swallowing
28. Allodynia- Hypersensitivity to touch that causes pain or discomfort
29. Apraxia – a disorder of movement characterised by the inability/difficulty to perform a voluntary movement despite an intact motor system. Can affect speech and movement. Can be described as ideomotor (difficulty in the ability to plan or complete a previously learned movement or mimic movements shown by someone else, despite adequate motor or sensory abilities) or ideational (Difficulties in executing a sequence of actions when performing a complex multi-task e.g. making a coffee
30. Akinesia- Inability to start a movement
31. Amnesia – Memory Loss
32. Ataxia – a lack of voluntary coordination of muscle movements, impairing their smooth performance. Also termed for a group of disorders that affect coordination, balance and speech.
33. Coordination – the ability to execute movement, which is accurate, smooth and controlled. This includes the appropriate speed, distance, direction, timing and muscular tension.
34. Bradykinesia- Slowness of movement
35. Festinating gait -commonly used to describe gait in PD. Characterised by a short stride length, increased speed of movement with tendency to lean forward.
36. Freezing – Abrupt termination of a movement/difficult initiating movement.
37. Nystagmus- Rhythmical oscillations of the eyes
38. Hemianopia- loss of half vision field
39. Diplopia- Double vision
40. VOR – Vestibulo-ocular Reflex (Reflex that controls eye gaze)

D. <u>Common abbreviations in case notes</u>

- DH – Drug History
- SH – Social History
- HPC – History of present condition
- PMH – Past Medical History
- UL – Upper limb
- LL – Lower limb
- Ax – Assessment
- Rx – Treatment
- S – Subjective
- O – Objective
- A– Analysis
- P - Plan
- RSW- Right sided weakness
- LSW- Left sided weakness
- LFD/RFD- Left facial droop/Right facial droop
- CKP – Central key point

- PKP – Proximal key point
- DKP – Distal key point
- COG – Centre of gravity
- BOS - Base of support
- Gait – The pattern of how an individual walks
- WZF – Wheeled Zimmer frame
- ZF – Zimmer frame
- W/S- walking stick
- CSW- Clinical support worker
- GF- Gutter frame
- 3WW/4WW – 3 and 4 wheeled walkers
- ROM – Range of movement
- AROM- Active Range of Movement
- PROM- Passive Range of Movement
- NAD – Nothing abnormal detected
- R – Right
- L – Left
- A02-assistance of 2
- STS- Sit to stand or stand to sit
- SOOB- Sit out of bed
- SOEOB- Sit on edge of bed
- W/C- Wheelchair
- AFO- Ankle Foot Orthosis
- PRAFO- Pressure relief ankle foot orthosis
- ADL- Activities of Daily Living
- PADL- Personal Activities of Daily Living
- DADL – Domestic Activities of Daily Living
- TUSS – Timed unsupported steady stand
- TUSS TOG – Timed unsupported steady stand with feet together
- SLS – Single leg standing
- D/C- Discharge
- POC- Package of care
- BD POC- 2x day care package
- TDS POC- 3x day care package
- QDS POC- 4x day care package
- A/W- Await
- MDT- Multidisciplinary Team
- TTOS – Medication on discharge